Public Digital Humanities and Trans Women's Healthcare: Exploring Migration, Government Schemes, and Social Advocacy in South India

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Introduction

Over the past fifteen years, multifaceted diversity, traditions, deep-rooted values, social customs, and regional practices have shaped India, prompting the development of trans women's healthcare and improved facilities and understanding. Health-beneficial schemes are available for trans women through both central and state governments. Therefore, they migrate to healthcare recognition locations within and outside India for advanced and specialised care (Padilla et al., 2016). The research explores the current landscape of healthcare and migration for South Indian trans women, identifying the benefits and barriers in central and state government schemes. However, individuals' challenges may vary from one location to another upon migration. It is crucial to examine the current status of trans individuals, as significant recent healthcare and migration advancements have been granted. The choice to focus on transgender people's healthcare migration destinations underscores the necessity of comprehensively understanding the contextual factors shaping their experiences.

Related Works

In the Indian context, researchers have conducted only a few studies, such as Nataraj (2022) and Acharya et al. (2023), to address the migration and healthcare access of the marginalised community. Their studies highlight how various forms of migration, such as economic, linguistic, medical, and kinship, intersect with gender transition and shape trans women's experiences. Raza and Karthikeyan (n.d) exemplify the dual nature of LGBTQ migration, emphasising both compulsion and choice in moving from small urban cities or rural areas to major centres. Their argument posits that individuals may resolve their identity issues out of compulsion but willingly migrate to pursue improved life opportunities. As Nataraj suggests, these migrations can provide access to better resources, supportive communities, and safer environments. However, trans individuals face challenges and barriers in different social, economic, and cultural contexts (Nataraj (2022) and Acharya et al. (2023)). Acharya et al. (2023) further emphasise that mental stress and the COVID-19 pandemic have severely impacted migrant trans women, reducing their livelihood opportunities.

To the authors' knowledge, no literature explores the migration and healthcare welfare schemes and the challenges of trans women in India. This study addresses key questions: 1. What challenges do trans women face when migrating to access medical facilities? 2. Do the beneficial medical facilities provided by state and central government schemes reach the trans women community, and how are these facilities utilised? 3. How can a digital platform enhance healthcare access, overcome geographical barriers, and contribute to the welfare of trans women?

This study examines the challenges trans women face at the intersection of migration and healthcare, focusing on medical, economic, and logistical barriers. It evaluates the accessibility and effectiveness of government healthcare schemes and explores the role of public digital humanities in improving social welfare, access to healthcare, and advocacy. The findings aim to enhance inclusion and empower trans women through digital initiatives.

Methodology

This study uses a mixed-method approach to explore Indian trans women's experiences with migration and healthcare. A purposive sample of 53 trans women participated in surveys and four interviews, focusing on challenges with government schemes like the Ayushman Bharat Scheme (2022), Transgender Welfare Board (2008), and Vidiyal Payanam Scheme (2021). Participants, recruited through convenience and snowball sampling from the Transgender Resource Centre, highlighted the barriers and progress in accessing healthcare and migration support in Madurai, Tamil Nadu.

Participant management

A team of five trans men, familiar with the Transgender Resource Center (TRC) and the trans women community, collected the data. They conducted 49 face-to-face surveys, while researchers completed four semi-structured four interviews and four surveys via phone, all administered in Tamil—the preferred language of both participants and investigators—to ensure clear communication and accurate responses. Surveys were

documented, and interviews were audiotaped (with consent) and transcribed into English. Data collection spanned five months (February–June 2024), with each one-hour session gathering detailed insights on migration and medical care at TRC, Madurai.

Ethical consideration

Although the investigators lacked formal ethical research training, their experience as transgender activists and social workers ensured respectful interactions with participants. Ethical guidelines, including informed consent, confidentiality, and data anonymity, were followed. A Google questionnaire was used for data collection reviewed by the TRC and an ethical review board. Participants were assured their access to state and central schemes would remain unaffected and were free to withdraw at any time. Comprehensive measures protected the identities and well-being of trans women throughout the study.

Discussion

This section briefly overviews gender-affirming facilities and migration challenges related to government schemes. Gender-affirming surgery (GAS) is prominent among South Indian trans women, as they seek to align their physical appearance with their gender identity. Most participants prefer private hospitals due to perceived higher quality of care, while only a small proportion benefit from state or central government schemes (See Table 1). A participant explained, “I know about the information of Tamil Nadu state scheme, but I do not know about central government schemes. I know both will take time and give me more tension, so I did not try both.” Lack of trust, limited knowledge of government systems, and reliance on informal networks hinder awareness and interest in healthcare schemes, which also leads to migration to access healthcare services.

In addition to these barriers, many trans women learn about schemes through friends and NGOs, emphasising their reliance on interpersonal networks. While these sources provide essential support, they can vary in accuracy. Social media offers broader access but faces mistrust due to language barriers and misinformation. Minimal family involvement, driven by societal stigma, further restricts access to information (See Table 1). To address these issues, improving outreach to families and ensuring accurate, accessible information through reliable channels like social media is crucial. Further, Tamil Nadu's free fare scheme for trans women significantly fosters mobility and confidence and challenges stereotypes. However, some participants (See Table 1) avoid using the scheme due to exclusions and overcrowding, limiting its impact. Issues such as infrequent services and discrimination underline the need for reforms to enhance trans women’s socio-economic development and integration into mainstream society.

These systemic barriers also push many trans women into precarious jobs like sex work and begging, though NGO initiatives have helped some find alternative livelihoods (See Table 1). Despite modest earnings (See Table 1), GAS remains a financial priority, exacerbating economic strain as government schemes do not cover treatments like silicone implants. A participant described spending significant savings on surgeries, highlighting their economic burden. Migration to urban areas for better opportunities is common, but transportation benefits offer limited relief, underscoring the need for effective policies and societal inclusivity.

Additionally, mental health counselling is mandatory before GAS in Tamil Nadu to mitigate post-surgery challenges. Despite this, systemic barriers persist, including counsellor bias, economic difficulties, and insufficient facilities. Many counsellors lack the skills to adequately support trans women, leading to mistrust and avoidance of care. As one participant noted, providers often imposed expectations that compounded feelings of discomfort and exclusion. NGOs like the Transgender Resource Center in Madurai have stepped in to bridge these gaps, connecting trans women with supportive healthcare providers. This role is crucial because, when doctors lack awareness of trans women’s specific needs, NGOs need to act as intermediaries to support trans individuals until healthcare providers gain the necessary understanding. Drawing on surveys and interviews with trans women, the study highlights the crucial role of awareness in reaching the broader trans-women community. It demonstrates that a lack of understanding leads to physical well-being and mental and economic stability challenges. The analysis emphasises that increased awareness can significantly improve medical and migration access.

Total (n=53)

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Variable N (%)

Interest Surgeries/Treatments Vaginoplasty 53 (100%)

Breast Augmentation Surgery 46 (86.80%)

Laser hair removal treatment 19 (35.80%)

Mental Health 52 (96.2%)

Access Statistics Vaginoplasty 46 (86.90%)

Breast Augmentation Surgery 44 (83%)

Laser hair removal treatment 18 (33.20%)

Mental health 52 (96.2%)

Hospital Accessed Government hospital 12 (21.70%)
Private hospital 31 (58.50%)
Not interested in revealing their healthcare choices 10 (19.80%)
Benefits from Scheme State scheme 10 (19.80%)
Central scheme 2 (3.80%)
Awareness of Scheme State scheme 48 (90.40%)
Central scheme 6 (11.8%)
Unawareness of Scheme State scheme 5 (9.60%)
Central scheme 47 (88.2%)
Sources of awareness Friends & NGOs 44 (83.02%)
Social media 7 (13.21%)
Family and relatives 2 (3.77%)
Monthly income of trans women 6,000 and 15,000 19 (35.7%)
15,000 to 18000 31 (58.3%)
18000 to 25,000 3 (6%)
Job Sex works 19 (36.5%)
Beggers 18 (34.6%)
Sex workers and Beggars 16 (30.8%)
Acting and modelling, floor cleaning, business, shopkeeping, and working in NGOs 13 (24%)
Savings from the free fare scheme 500 to 1000 30 (57.1%)
1000 to 2500 23 (42.9%)
Free fare scheme Increased visibility and safety 47 (88.2%)
Avoid due to bus restrictions 6 (11.8%)
Table 1. Address the data on gender-affirming healthcare, economic conditions, and support scheme awareness among trans women (n=53) in Madurai, Tamil Nadu.

Conclusion

To enhance the socio-economic development of trans women, policymakers must ensure that beneficial schemes comprehensively reach all segments of society. Increased promotion, awareness, and advancements in these schemes are necessary to meet the needs of the trans women community. Additionally, extending free bus fare facilities specifically to trans women is recommended to improve their healthcare access, mobility, economic opportunities, and integration into mainstream society.

Future work

Future investigations will broadly explore the impacts, challenges, and developments of state and central healthcare schemes, focusing on trans women's migration experiences, barriers, and successes in India. In addition to these areas, the study will centre around three key research questions. Moreover, in the realm of Public Digital Humanities, the study aims to improve access to trustworthy information by proposing a collaborative platform using Google Maps, where trans women can share their healthcare experiences. Users can update details such as issues faced, benefits, distance, pricing, and service quality. This user-driven approach will provide reliable insights, empower informed decisions, and enhance healthcare access.

Furthermore, providing this information will help trans women access treatment near their new locations, ensuring quality care and overcoming geographical barriers. Additionally, NGOs and trans women can use this platform to guide their healthcare needs, access relevant healthcare policies, and raise awareness about accessible healthcare practices. The Google Maps application will serve as a supportive community network where individuals can share experiences, seek advice, and offer mutual assistance.

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